

1. Administrative & Study Identification

Full Study Title: A Randomised, Double-blind, Placebo-controlled, Phase III Study of Olaparib Maintenance Monotherapy in Participants With BRCA Wild Type Advanced High Grade Serous or Endometrioid Ovarian Cancer Following Response to Standard First-line Platinum-based Chemotherapy **Short Title/Acronym:** MONO-OLA1 (D9319C00001- 1L OC Mono Global RCT) **Protocol Number:** D9319C00001 **NCT Number:** NCT04884360 **Posting ID:** 742712228764732066 **Sponsor/ Company Name:** AstraZeneca **Investigational Product:** Olaparib (Trade Name: Lynparza) **ATC Code:** L01XK01 **EMA URL:** [Link](#) **Phase of Development:** Phase III (Criteria Type: PHASE_REQUIREMENT - Trial must be in PHASE3) **Trial Status:** ACTIVE_NOT_RECRUITING (Active but not currently recruiting) **Disease Area / Preferred Name:** Ovarian Cancer **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the efficacy and safety of olaparib maintenance monotherapy compared with placebo by evaluating progression-free survival (PFS) in patients with BRCA wild-type (BRCAwt), homologous recombination deficiency (HRD)-positive advanced ovarian cancer, and in all patients with BRCAwt disease. **Secondary Objectives:** To evaluate Overall Survival (OS), second Progression-Free Survival (PFS2), Time to First Subsequent Therapy or death (TFST), Time to Second Subsequent Therapy or death, health-related quality of life (HRQoL), as well as overall safety and tolerability.

2.2 Background & Rationale While poly(ADP-ribose) polymerase (PARP) inhibitors have demonstrated significant clinical benefits in the first-line maintenance treatment of newly diagnosed advanced ovarian cancer (OC), viable treatment options for patients with BRCA wild-type disease remain a critical unmet medical need. Given that BRCA wild-type disease presents in approximately 60% to 80% of patients with high-grade serous ovarian cancer, the MONO-OLA1 study is designed to explore the clinical benefit of maintenance olaparib in this specific patient population who have responded to first-line platinum-based chemotherapy.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** 2:1

(Olaparib vs. Placebo), stratified by response to 1L platinum-based chemotherapy (CR vs. PR), HRD status, and region.

3.2 Planned Enrollment & Duration Targeted Enrollment: 366 patients **Number of Sites:** Global trial across multiple international sites **Estimated Study Start:** 31-May-2021 **Estimated Primary Completion:** 27-Jul-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Female participants ≥ 18 years of age at the time of screening. 2. Histologically newly diagnosed high-grade serous or high-grade endometrioid ovarian cancer, fallopian tube cancer, or primary peritoneal cancer that is Stage III or IV according to International FIGO 2014 criteria. * *Stage III*: primary debulking surgery with macroscopic residual disease post-surgery, neoadjuvant chemotherapy, or inoperable. * *Stage IV*: primary debulking surgery regardless of residual disease, neoadjuvant chemotherapy, or inoperable. 3. Provision of a formalin-fixed, paraffin-embedded (FFPE) tumour sample for central testing. Documented BRCA wild-type (BRCAwt) advanced ovarian cancer via central testing is required prior to randomisation. 4. Patients must be in clinical Complete Response (CR) or Partial Response (PR) per RECIST 1.1 following 6 to 9 cycles of standard first-line platinum-based chemotherapy (or a minimum of 4 cycles if discontinued early due to platinum-related toxicities), with no measurable lesion > 2 cm on post-treatment scan. 5. Pre-treatment CA-125 measurements in the normal range or decreased by $\geq 90\%$ during front-line therapy and stable for at least 7 days. 6. Randomized within a maximum of 12 weeks from the last day of chemotherapy infusion (no earlier than 3 weeks). 7. ECOG performance status of 0 or 1 with no deterioration over the previous 2 weeks prior to randomisation. 8. Adequate organ and marrow function.

4.2 Exclusion Criteria 1. Participants with stable disease (SD) or progressive disease (PD) on the post-treatment scan, or any evidence of progression prior to randomisation. 2. Mucinous or clear cell subtypes of epithelial ovarian cancer, carcinosarcoma, undifferentiated ovarian cancer, non-epithelial ovarian cancer, borderline tumours, or low-grade epithelial ovarian tumours. 3. Participants with Stage III disease who have had complete cytoreduction (i.e., no macroscopic residual disease) at their primary debulking surgery. 4. Participants who have undergone ≥ 2 debulking (cytoreductive) surgeries. 5. History of another primary malignancy (exceptions include adequately treated curable diseases ≥ 5 years ago with low recurrence risk, such as non-melanoma skin cancer or localised TNBC treated > 3 years ago). 6. Persistent toxicities (CTCAE Grade ≥ 2) caused by previous anticancer therapy (excluding alopecia and Grade 2 peripheral neuropathy). 7. Participant is immunocompromised. 8. Prior exposure to a PARP inhibitor, including olaparib. 9. Receipt of

bevacizumab with first-line chemotherapy or planned administration of bevacizumab maintenance therapy. 10. Any concurrent anticancer treatment. 11. Currently pregnant or breast-feeding.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Olaparib (L01XK01) 300 mg tablets orally twice daily (BID), or matching placebo tablets orally twice daily (BID). **Route of Administration:** Oral. **Duration of Treatment:** Up to 24 months (2 years) or until objective radiological disease progression per RECIST 1.1, unacceptable toxicity, or withdrawal of consent.

5.2 Concomitant & Prohibited Therapy Permitted: Standard concomitant medications for symptomatic management and supportive care per investigator discretion. **Prohibited:** Concurrent administration of other investigational anti-cancer agents, bevacizumab maintenance therapy, or other targeted anti-cancer therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, BRCA/HRD testing (FFPE sample), Baseline tumor imaging, Medical History
Baseline	Day 0	Randomization (2:1 within 12 weeks of last chemo), First Dose Dispensation, Vital Signs
Interim	Every 4 to 8 Weeks	Safety Labs, Efficacy Assessment (RECIST 1.1 scans), HRQoL Questionnaires, IP Accountability
End of Study	Up to 24 Months	Final safety/efficacy assessments, survival follow-up, Exit Interview

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Superiority of olaparib maintenance treatment relative to placebo by assessment of Progression-Free Survival (PFS) in the BRCAwt HRD-positive subgroup, and PFS in the overall BRCAwt patient population. **Measurement Method:** Defined as time from randomisation until progression per RECIST 1.1 as assessed by the investigator at the local site, or death due to any cause.

7.2 Secondary Endpoints 1. Superiority of olaparib maintenance treatment relative to placebo by assessment of Overall Survival (OS). 2. Second Progression-Free Survival (PFS2). 3. Superiority of olaparib maintenance treatment relative to placebo by assessment of Time to First Subsequent Therapy (TFST) or death. 4. Time to Second Subsequent Therapy or death. 5. Health-related Quality of Life (HRQoL) and overall safety profiles.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous collection via patient clinical visits, routine laboratory assessments, and patient pill diaries. **Serious Adverse Events (SAEs):** Must be reported to the Sponsor and regulatory authorities within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** An Independent Data Monitoring Committee (IDMC) will review ongoing safety and efficacy data periodically.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for all primary and secondary efficacy endpoints; Safety Set for AE/SAE evaluations. **Sample Size Justification:** A targeted sample size of exactly 366 patients randomly assigned 2:1 is powered to detect a statistically significant difference in PFS. The number of patients with HRD-negative/unknown status will be capped at a specific subset of the total patient population. **Handling of Missing Data:** Standard survival analysis techniques including right-censoring at the date of the last evaluable tumor assessment for patients without documented progression.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular on-site and remote monitoring visits to ensure protocol adherence, data integrity, and patient safety. **Audit Trail:** Validated eCRF systems with complete audit trails, data clarification form tracking, and blinded independent central review for PFS sensitivity analysis.

11. Study Identifiers & Governance

Primary ID: D9319C00001 **Registry Number:** NCT04884360 **Posting ID:** 742712228764732066 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** MONO-OLA1 (D9319C00001- 1L OC Mono Global RCT) **Official Regulatory Title:** A Randomised, Double-blind, Placebo-controlled, Phase III Study of Olaparib Maintenance Monotherapy in Participants With BRCA Wild Type Advanced High Grade Serous or Endometrioid Ovarian Cancer Following Response to Standard First-line Platinum-based Chemotherapy

12. Clinical Framework (Disease Specific)

Primary Condition: Ovarian Cancer (Stage III/IV) **Diagnosis Threshold:** Newly diagnosed high-grade serous or endometrioid ovarian cancer, BRCA wild-type (BRCAwt) **Medical Methodology:** 1L Platinum-based chemotherapy complete or partial response followed by 1L Maintenance Therapy **Optimization Target:** Progression-Free Survival (PFS) and delayed time to subsequent therapies

13. Study Procedures & Methodology

Management Pathway: Standard oncology clinical pathway for 1L maintenance in ovarian cancer **Education Protocol (HCP):** Protocol-specific training, Good Clinical Practice, and site initiation visits **Education Protocol (Patient):** Study-specific dosing guidelines, pill diaries, and toxicity self-management instructions **Quality Audit Mechanism:** Clinical study monitoring, independent safety oversight, and independent central imaging review

14. Observation & Follow-Up Schedule

Total Duration: Up to 24 months of treatment plus long-term survival follow-up **Onsite Visit Cadence:** Every 4 to 8 weeks for scans and clinical assessment **Remote Monitoring Frequency:** Interim phone contacts and continuous safety reporting **Checklist Review Interval:** Continuous AE/SAE tracking and Investigational Product (IP) compliance review at every visit

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				